

Sachetana v2: A Briefing for the Review Panel

An overview of the proposed platform, the population it serves, and the rationale for developing it within the university. This briefing addresses product rationale only; pricing and budget are addressed separately.

Prepared for **MAHE, discovery discussion (review panel)**

Date **14 September 2026**

I. Background and Rationale

Most students who require mental health support do not seek it.

In 2023, 13,892 student suicides were recorded in India, the highest count in a decade (National Crime Records Bureau, 2023). Approximately 13 percent of students use on campus counselling services, although most students report their own mental health as only fair or poor (College Pulse, 2022).

II. The Regulatory Mandate

Since July 2025, this has been a legal obligation rather than an optional initiative.

A Supreme Court order requires every Indian college and university to implement a specific set of measures for its students. These are not recommended practices; they constitute a legal obligation with defined, named ratios.

Under the Court's order, every institution is now required to provide:

- One qualified counsellor at any institution with 100 or more students
- A mental health and wellbeing centre on campus
- Peer supporters at a ratio of 1 to 100 students, and faculty mentors at a ratio of 1 to 500
- A 24 hour helpline integrated with the national Tele-MANAS crisis service
- Staff training delivered twice annually by certified professionals

MAHE has already established the clinical infrastructure this requires. What remains is the connective structure between a student recognizing a concern and that student being seen by a clinician the university already employs.

III. Competitive Landscape

The market includes multiple platforms; none addresses the specific need MAHE has.

Several existing platforms address portions of this problem. None addresses the function MAHE actually requires.

Consumer wellness applications. Meditation, mood tracking, and guided content. These are widely used and well built, but are not connected to any institution or clinician. Examples: Calm, Headspace, Wysa.

Pay-per-session platforms. These platforms supply their own therapists and are paid per session, which gives them no incentive to refer a student to an external provider. Examples: BetterHelp, Talkspace.

Campus platforms operating abroad. The closest match in structure, though built for American universities, American law, and American billing systems. Examples: TimelyCare, Uwill, Mantra Health.

None of these platforms is designed to direct a student toward the psychiatrists and counsellors that their own university already funds. That is the unaddressed need this proposal is designed to meet.

IV. Proposed Solution

Sachetana is not a replacement for MAHE's clinical care; it is the pathway to it.

A student who would not otherwise approach the counselling centre directly can begin using Sachetana instead, to be heard, directed appropriately, and, where warranted, connected to the same clinicians MAHE already funds.

Step 1, the student. A student notices a concern but does not know whom to approach, or whether the concern warrants attention.

Step 2, Sachetana as intermediary. Sachetana provides a brief, private initial step: a guided conversation that clarifies an appropriate next step.

Step 3, MAHE's existing care. Kasturba Hospital, the Department of Psychiatry, and salaried counsellors already in place.

V. Proposed Feature Set

Each of the following five features addresses a specific stage of the student's interaction with the platform, from first contact through clinical escalation.

1. Private initial access

A brief, simple sign-in process, without public visibility or unnecessary disclosure, followed by a guided conversation with an AI assistant that identifies an appropriate form of support. A short, validated screening instrument provides further clinical context. Because visibility itself is a barrier, a discretion control allows the interface to be replaced instantly with an unrelated screen.

Addresses: Not knowing whom to approach, or whether a concern is sufficiently serious to raise.

2. Ongoing self-directed support

Mood tracking, journaling, and a library of guided self-help content, built without streaks, badges, or leaderboards, in recognition that gamified engagement mechanisms are inappropriate for users already experiencing distress.

Addresses: The interval between an initial conversation and a scheduled appointment, which may extend to several weeks.

3. Peer and community support

Moderated peer support under continuous clinical supervision, supported by an offline peer-supporter network. This directly addresses the Supreme Court's 1-to-100 peer-supporter requirement, implemented as a managed system rather than an unstructured arrangement.

Addresses: Isolation, which can convert a difficult period into a crisis.

4. Clinical scheduling

Direct scheduling into MAHE's existing counselling staff, with a defined escalation pathway to the Department of Psychiatry when a student's needs exceed what the platform can address.

Addresses: The two-to-eight week waiting period that currently separates a request for help from an appointment.

5. Crisis response

A single control connects the user directly to Tele-MANAS, India's 24-hour national mental health crisis line, without intermediate steps.

Addresses: The immediate minutes following a crisis, when timely response matters most.

Tools for MAHE's clinical and administrative staff

- A counsellor dashboard consolidating caseloads, clinical notes, and triage flags into a single interface, rather than distributing this information across email and paper records.
 - Automated generation of compliance evidence: the ratios and records required under the Court's order, produced as a byproduct of ordinary platform use.
 - Multilingual support in Hindi and regional languages, in addition to English, consistent with Tele-MANAS's existing language coverage.
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VI. Confidentiality Rather Than Anonymity

The platform requests identification rather than removing it, and this is a deliberate design decision.

Some platforms permit use without any identification. Sachetana requires a student to sign in. This is an intentional design choice.

The cost of anonymity. Without identification, no appointment can be scheduled for a specific individual, no follow-up is possible, and there is no means of confirming that a crisis flag reached an actual person. Anonymity can protect an initial message, but it cannot protect the student afterward.

The benefit of confidentiality. A verified identity, protected under the same legal standard that governs any patient's psychiatric record: never sold, never provided to advertisers, and accessible only to individuals directly involved in that student's care.

VII. Rationale for This Approach

Two structural factors explain why this has not previously been built.

1. Commercial platforms are incentivized to retain the student.

Commercial platforms generate revenue directly from therapy sessions, which creates an incentive to retain each student within their own network. MAHE's incentives run in the opposite direction: its clinicians are already salaried, and the institution benefits from filling their calendars rather than an external provider's.

2. The peer-supporter mandate remains unaddressed.

A 1-to-100 peer-supporter ratio requires recruiting, training, and managing several hundred individuals at MAHE's scale. No platform currently on the market provides this capability. It represents one of the largest gaps between the current legal requirement and any existing product.

VIII. Requested Panel Input

This section outlines three requests, none of which requires a numeric commitment.

1. Approval of the overall direction

Confirmation that the platform should function as a bridge into MAHE's existing clinical care, rather than as a stand-alone wellness application.

2. Guidance on sequencing

An indication of which of the five proposed features the panel considers the highest priority to implement first.

3. Identification of additional stakeholders

Confirmation of which additional departments, including Student Affairs, the Department of Psychiatry, and IT and Security, should be involved before the project proceeds further.

Prepared for MAHE's discovery discussion, 14 September 2026. This briefing addresses product rationale only. Competitor research and the legal and compliance review are documented separately and available on request.